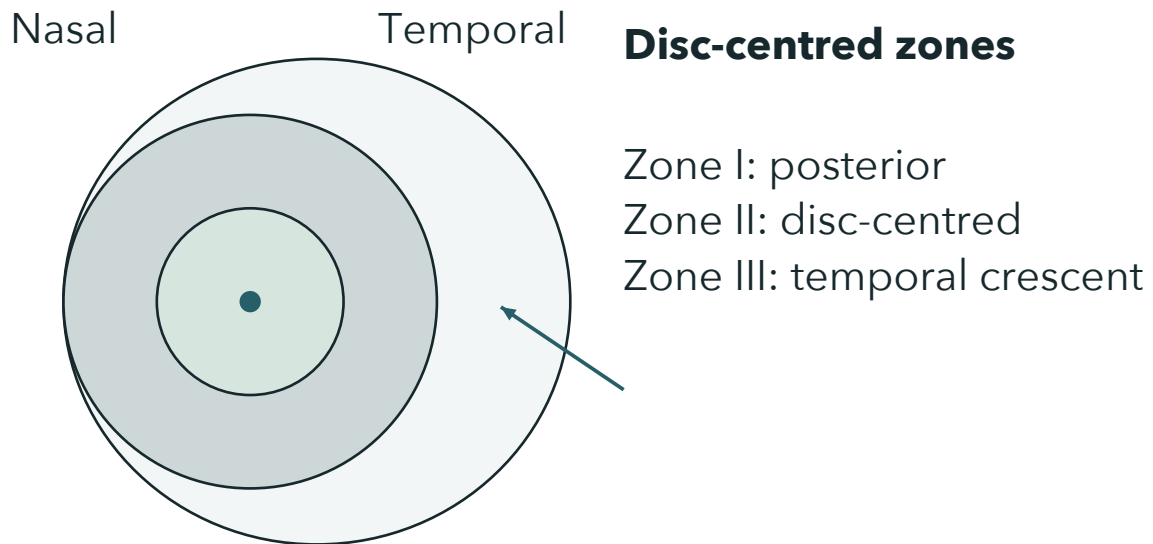


ROP modality and late follow-up

Part 2 | Describe ROP precisely

Zone, stage and vascular activity guide a treatment discussion.



Original simplified map: zone III is a temporal crescent, not a complete annulus.

Record the location, extent, stage and plus-spectrum findings using the current classification. Aggressive ROP may not follow a tidy sequential progression, and treatment cannot be selected by stage alone. Describe posterior disease and vascular activity before comparing modalities. The examination response should make clear which observed feature changes urgency rather than merely reproducing a stage list.

Laser versus anti-VEGF

Different interventions create different residual anatomy.

Finding	Implication
Laser	Ablates avascular peripheral retina
Anti-VEGF	Suppresses the angiogenic drive; vascularisation may remain incomplete
Posterior or aggressive disease	Modality requires specialist selection
Family counselling	Include retreatment and prolonged surveillance

Compare the eye's zone, activity, media and systemic condition with the treatment options. Anti-VEGF does not mean the peripheral retina is instantly mature, while laser has permanent anatomical consequences. Explain how follow-up capacity affects safety. Neither a rapid visible response nor the convenience of an injection settles the long-term decision.

BEAT-ROP: read the zone and the limitations

A study result for posterior disease is not universal superiority.

Finding	Implication
Comparison	Bevacizumab versus laser in selected severe ROP
Benefit signal	Recurrence advantage concentrated in zoneI
Limits	Different zones and systemic safety require separate interpretation
Practice translation	Use current guidance rather than one historical headline

Identify the population, endpoint and follow-up before using the trial to counsel a family. A reduction in recurrence is not proof of equivalent long-term systemic safety or better outcomes for every zone. State what the study answers and what remains uncertain, then connect the decision to current specialist protocols.

RAINBOW: long-term outcomes add information

Five-year results support a more detailed benefit-risk conversation.

Finding	Implication
Comparison	Ranibizumab strategies versus laser
Visual acuity	Similar measured vision at five years
Refractive outcome	Less high myopia in ranibizumab groups
Systemic interpretation	No detected difference does not prove zero risk

Keep visual, refractive and developmental endpoints separate. A study without a detected nonocular safety difference cannot exclude every rare or subtle harm. Explain which outcomes matter to the family and acknowledge the limits of follow-up size and measurement. Do not transfer a result for one agent directly to every intravitreal anti-VEGF product.

Regression is not the end of surveillance

Reactivation can occur after an initially reassuring response.

- Document regression and remaining avascular retina
↓
- Schedule modality-specific specialist surveillance
↓
- Assess reactivation and completion of vascularisation
↓
- Continue later visual, refractive and alignment care

Families need a written follow-up plan before discharge or transfer. A lapse after anti-VEGF can miss late reactivation, so early regression is not a reason to stop examinations. Keep neonatal retinal surveillance distinct from subsequent monitoring of myopia, amblyopia and strabismus. Each addresses a different risk and requires a clear handover.